GAMSA · Gender-Affirming Medicines South Australia

Competency assessment checklist

Ventrogluteal intramuscular administration of testosterone undecanoate 1000 mg/4 mL

DOCUMENT ID	VERSION	EFFECTIVE	REVISED	NEXT REVIEW
GAMSA-TU-COMP	2.6	5 October 2025	7 November 2026	24 October 2027, or on any change to the Product Information

Current version

Prepared by Theodore South, BPharm (Hons), registered pharmacist, Adelaide. Questions or corrections: gamsaenquiries@gmail.com. A template for adaptation to local policy, offered freely; not endorsed by any health service. Clinics remain responsible for their own procedures, credentialling and emergency arrangements. Where this document and the Product Information differ, the Product Information prevails [1].

Assesses competency in the safe preparation and administration of testosterone undecanoate 1000 mg/4 mL by deep intramuscular injection at the ventrogluteal site. Every criterion must be demonstrated under direct observation (2a is a recommended step: tick N/A if pregnancy is not possible); the SOP and training guide sets the standard expected. Complete after at least three supervised injections.

Clinician	Position	Assessor	Date

Competency criteria

#	Criterion	Source	Competent	Comments
1	Confirms patient identity with at least three approved identifiers (not sex or gender) and matches them to the prescription, using the name the person uses; confirms allergies, contraindications and informed consent, including any prior reaction to an oil-based injection	[1][12][13]	☐ Yes ☐ No	
1a	Uses the person's name and pronouns, explains each step and asks before touching, offers a chaperone or support person, uncovers only the injection site, and arranges a professional interpreter if needed	[12][13][17]	☐ Yes ☐ No	
2	Screens for current chest infection or breathlessness, previous POME, and bleeding or coagulation disorder or anticoagulant use; knows to defer or refer	[1][11]	☐ Yes ☐ No	
2a	Recommended: if the patient could become pregnant, checks that a pregnancy test has been done before the injection; knows testosterone is contraindicated in pregnancy	[1][12]	☐ Yes ☐ No ☐ N/A	
3	Confirms product, dose (1000 mg in 4 mL), expiry and batch number	[1][2]	☐ Yes ☐ No	
4	Prepares equipment: 18G drawing-up needle, 20–22G injection needle of suitable length (38 mm; 50 mm if BMI over 30), 5 mL syringe, antiseptic, dressing, sharps container	[5][10]	☐ Yes ☐ No	
4a	Cleans hands at the 5 Moments (before touching the patient, before the injection, after the injection or any blood exposure, after touching the patient and their surroundings) and puts on gloves before the injection	[5][14]	☐ Yes ☐ No	
5	Confirms emergency equipment is available and in date: oxygen, adrenaline, monitoring, means of calling 000	[8]	☐ Yes ☐ No	
6	Positions the patient and landmarks the ventrogluteal site correctly (greater trochanter, ASIS, iliac crest; centre of the V)	[4][5]	☐ Yes ☐ No	

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#	Criterion	Source	Competent	Comments
7	Inserts the needle at 90° to a depth that reaches muscle; aspirates, and knows to withdraw and re-site if blood returns	[5][10]	☐ Yes ☐ No	
8	Injects the full 4 mL very slowly, over approximately 2 minutes	[1][3][10]	☐ Yes ☐ No	
9	Withdraws the needle, applies gentle pressure without massage, applies dressing, disposes of sharps immediately	[5]	☐ Yes ☐ No	
10	Observes the patient during and immediately after the injection, and for at least 30 minutes under this procedure (POME typically presents within 30 minutes; the PI gives no fixed time); books the next appointment	[1][3][12]	☐ Yes ☐ No	
11	Describes the signs of POME and of anaphylaxis, and the immediate management of each, including adult adrenaline dose, route and repeat	[6][7][8]	☐ Yes ☐ No	
12	Documents product, batch, expiry, dose, side, needle, clinician, observation outcome and next due date; knows how to report to the TGA (AEMS)	[1][9]	☐ Yes ☐ No	
12a	Describes what to do after a needlestick or blood splash: wash or rinse straight away, tell the manager and record it, get medical advice straight away	[15][16]	☐ Yes ☐ No	

Assessment outcome

☐ **Competent**	☐ **Requires further supervised practice**	☐ **Not yet competent**

Assessor comments and plan:

Reassessment: every 12 months, and sooner after a POME or anaphylaxis event at the clinic, or if the clinician has not given this injection for 6 months.

Reassess by (date)	Reason, if sooner	Reassessed on (date)

Assessor signature	Name (print)	Date

Clinician signature	Name (print)	Date

Sources Numbered as in the SOP and training guide, where full references are given: [1] Reandron 1000 Australian PI, Aspen, rev. Dec 2025 · [2] Reandron 1000 ARTG 213942 · [3] Reandron 1000 NZ data sheet, Dec 2025 · [4] SA Health ventrogluteal brochure · [5] SESLHD GL/076 v2.6, 2025 · [6] Pastuszak 2020 (POME safety analysis) · [7] Adam 2023 (POME review) · [8] ASCIA anaphylaxis guideline, October 2026 · [9] TGA AEMS adverse event reporting · [10] UK administration guidance (Liconsa), 2024 · [11] Testosterone ADVZ 1000 Australian PI · [12] AusPATH Australian Standards of Care, 2026 · [13] Canberra Health Services CHS25/344 · [14] ACSQHC hand hygiene guide, 2023 · [15] NSW Health GL2024_002, 2024 · [16] NIOSH 2012-123 · [17] TIS National, 131 450. Current package: https://gamsa.au/medicines/testosterone-undecanoate

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